Adult intravenous fluid resuscitation

Hypovolaemia suggested by:

- Clinical examination
- Trends in observations (inc. urine output)
- Clinical picture

Indicators that patient may need fluid resuscitation

- Systolic BP < 100mmHg
- Heart rate > 90bpm
- CRT > 2 sec
- Cold peripheries
- Resp rate > 20 breaths/min
- EWS ≥ 5
- 45° passive leg raise suggests fluid responsiveness

Types of shock:

Hypovolaemia e.g.
- Blood loss – any source
- Gastrointestinal loss
- Trauma
- Ruptured ectopic

Distributive e.g.
- Sepsis
- Anaphylaxis
- Neurogenic

Obstructive
- Massive pulonary embolism
- Tension pneumothorax
- Cardiac tamponade

Cardiogenic
- Primary pump failure
- Massive MI

IV Fluid Therapy

Click the drip for more information

On-going blood loss – GET HELP!
Attempt to control the bleeding
Massive transfusion policy may need to be activated.

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Resuscitation Algorithm (NICE)

Resuscitation Fluid

Crystalloid containing 130-154mmol/l Na e.g. 0.9 % Sodium chloride, Plasma-Lyte

Assess patient ABCDE
Is your patient hypovolaemic?
Monitor Patient's Observations

No → **Consider routine fluid requirements**

Yes ↓

- **Identify cause of hypovolaemia and treat**
- **Fluid bolus 500mls over max 15min (250mls if risk of Cardiac failure)**

↓

Reassess ABCDE
Is further Fluid resuscitation needed?

Yes ↓ **>2000mls given**

- No ↓ **Further bolus 250-500mls over max 15 mins** → (back to Reassess ABCDE)
- Yes ↓ **Senior Help**

No ↓ **Is your patient still showing signs of shock?**

- Yes ↓ **Senior Help**
- No ↓ **Consider patient's ongoing IV fluid need & electrolytes**

After each bolus, do you need help?

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